

1. A 56 year-old man presented with progressive abdominal distension and leg swelling of two months duration. He is a known patient with cirrhosis for the past one year on follow up but not on medication. On examination, he was not in distress and has moderate ascites and bilateral leg swelling. Which of the following is the best initial management?

- A. Furosemide
- B Furosemide and Spironolactone
- C. Hydrochlorothiazide
- D. Paracentesis and Furosemide

2. A 40 year-old man is brought to the Emergency Department because he was vomiting fresh blood. He reports that he has no abdominal pain or nausea. On physical examination; BP-70/40 mmHg, PR=118/min, RR= 24/min. He also has gynecomastia, dilated abdominal veins and ascites. Which of the following is the most likely cause of the bleeding?

- A Esophageal cancer
- B. Esophageal varices
- C. Gastric cancer
- D. Reflux esophagitis

3. A 50 year-old woman brought an ultrasound imaging result which showed Gall Bladder Stone. She has no abdominal pain and no known medical illness. Which of the following is the recommended management?

- A. Cholecystectomy
- B. Extracorporeal Shockwave Lithotripsy (ESWL)
- C. Reassurance
- D. Ursodeoxycholic acid

4. A 68 year-old man is admitted to a Hospital with complaint of intermittent lower abdominal pain, bowel habit change and rectal bleeding of one month duration. He has tenesmus incomplete sense of defecation and had lost weight. What is the most likely diagnosis?

- A. Amebic dysentery
- B. Colorectal cancer
- C. Sigmoid Volvulus

D. Ulcerative colitis

5. A 23 year-old farmer presented to the OPD with abdominal pain and frequent vomiting of one day duration. On examination the abdomen is slightly distended and has visible peristalsis. Plain abdominal x-ray showed step ladder pattern air-fluid level. Which of the following is the most likely diagnosis?

A. Caecal Volvulus

B. Compound Volvulus

C. Sigmoid Volvulus

D. Small bowel Volvulus

6. A 30 year-old twin pregnant woman was admitted to a surgical ward with Cholelithiasis. She is obese and had repeated history of the disease. She is on low-dose estrogen therapy. What is the most likely etiology for the above diagnosis?

(A) Cystic fibrosis

(B) Pigment gallstones

(C) Cholesterol gallstones

(D) Acalculous inflammation

7. A 32-year-old male patient came to a hospital with right upper quadrant pain and tenderness, fatigue, bloody vomiting and bright yellow jaundice. After laboratory investigation of liver function tests, a nurse confirmed that the problem is Hepatitis B virus infection. What is the most appropriate laboratory finding that confirms the diagnosis?

(A) SGPT(ALT) =9UL

(B) Albumin =5.6 g/dL

(C) SGOT(AST) =105 U/L

(D) Total Bilirubin=1.1 mg/dL

8. In routine morning care for client with colostomy, you observed that content coming from colostomy opening was fluid. The patient reported that he always has similar appearance of discharge. What is the most likely type of colostomy in this patient?

(A) Sigmoid colostomy

(B) Ascending colostomy

(C) Transverse colostomy

D) Descending colostomy

9. A 29-year-old male patient was scheduled for emergency surgery for management of acute abdomen secondary to appendicitis. A pre operative nurse needs to prepare the patient for surgery. What psychological component should be assessed first?

(A) Level of understanding for pre & postoperative instructions

(B) Coping strategies for common health problems

(C) Emotional state related to current diagnosis

(D) Roles and responsibilities at home

10. A male prisoner patient came to a client with the some complaint of abrupt onset of fever, malaise, anorexia, and abdominal discomfort. He has visible jaundice, He strongly criticizes the prison site for the poor hygiene of the drinking water. The lab finding showed that his IgM has abnormally increased. What is the possible diagnosis for this client?

(A) Hepatitis A

(B) Hepatitis B

(C) Hepatitis C

(D) Hepatitis D

11. A patient starts to experience pain while receiving an enema. The nurse notes blood in the return fluid and rectal bleeding. What action does the nurse take first?

A. Administers pain medication

B. Slows down the rate of instillation

C. Tells the patient to breathe slowly and relax

D. Stops the instillation and obtains vital signs

12. A nurse is caring for a client who was admitted to the hospital with upper gastrointestinal bleeding. For which clinical indicator associated with upper gastrointestinal bleeding should the nurse assess the client?

A. Pale, clay-colored stool

B. Yellow, greenish stool

C. Hard, dry brown stool

D. Black, tarry stool

13. A tap-water enema is prescribed for a client. The client says, "How does this type of enema work?" Which rationale should the nurse include in words the client will understand when answering the client's question?

- A. Water causes excessive interstitial fluid loss.
- B. Instilled water stimulates a bowel movement.
- C. Surface tension of water is reduced by soaps.
- D. Hypertonic nature of the water irritates the intestinal mucosa

14. The nurse listens for bowel sounds before administering an enema. The patient asks, "Why are you listening to my abdomen?" The most accurate response for the nurse to make is?

- A. "The presence of bowel sounds indicates the presence of peristalsis."
- B. "I am listening to make sure that you need the enema."
- C. "I am listening to determine which position I should place you in for the administration of the enema."
- D. The presence of bowel sounds indicates you will be able to hold the solution."

15. A nurse is evaluating a postoperative patient and notes a moderate amount of serous drainage on the dressing 24 hours after surgery. Which of the following is the appropriate nursing action?

- A. Notify the surgeon about evidence of infection immediately.
- B. Leave the dressing intact to avoid disturbing the wound site.
- C. Remove the dressing and leave the wound site open to air.
- D. Change the dressing and document the clean appearance of the wound site.

16. A nurse cares for a patient who has a nasogastric tube attached to low suction because of a suspected bowel obstruction. Which of the following arterial blood gas results might be expected in this patient?

- A. pH 7.52, PCO₂ 54 mmHg.
- B. pH 7.42, PCO₂ 40 mmHg.
- C. pH 7.25, PCO₂ 25 mmHg.
- D. pH 7.38, PCO₂ 36 mmHg.

17. The nurse is educating a patient with liver cirrhosis about dietary and lifestyle changes. Which statement by the patient indicates a need for further teaching?

- A. "I will avoid alcohol completely to prevent further liver damage."
- B. "I will try to eat more foods high in protein to build strength."
- C. "I need to restrict my salt intake to help control fluid buildup."
- D. "I will monitor my weight daily to check for fluid retention."

18. A 65-year-old patient is admitted with abdominal pain, vomiting, and a history of constipation. The nurse notes abdominal distension and hyperactive bowel sounds in the upper quadrants but absent bowel sounds in the lower quadrants. Which of the following actions should the nurse take first?

- A. Administer pain medication as prescribed.
- B. Prepare the patient for surgery.
- C. Insert a nasogastric (NG) tube for decompression.
- D. Encourage the patient to drink fluids.

19. A 20-year-old patient presents to the emergency department with complaints of abdominal pain that began around the umbilicus and has now moved to the lower right quadrant. Which additional symptom would most strongly suggest appendicitis?

- A. Nausea and vomiting
- B. Rebound tenderness in the right lower quadrant
- C. Dull, generalized abdominal pain
- D. Bright red blood in the stool

20. A client with GERD reports frequent heartburn and regurgitation. Which nursing diagnosis is the priority?

- A. Risk for Aspiration.
- B. Imbalanced Nutrition: Less than Body Requirements.
- C. Deficient Knowledge related to lifestyle modifications.
- D. Acute Pain related to esophageal irritation.

21. A patient with an intestinal obstruction is scheduled for surgery in a few hours. Which nursing intervention would be the priority before surgery?

- A. Administering the ordered oral laxative
- B. Placing the patient in Fowler's position
- C. Providing the patient with ice chips for comfort
- D. Inserting an indwelling catheter

22. The nurse is caring for a patient with peritonitis. Which of the following interventions should the nurse prioritize?

- A. Encourage oral fluid intake
- B. Position the patient in a supine position
- C. Monitor vital signs
- D. Apply a warm compress to the abdomen

23. A patient with liver cirrhosis is receiving lactulose to treat hepatic encephalopathy. Which of the following outcomes would indicate that the medication is effective?

- A. Patient has one soft stool per day

- B. Patient reports no abdominal pain
- C. Decreased confusion and improved mental status
- D. Increased serum ammonia levels

24. A patient is admitted with severe right upper quadrant (RUQ) pain, nausea, and vomiting. The pain radiates to the right shoulder and increases after eating a fatty meal. The nurse suspects cholelithiasis. Which additional symptom would most likely support this diagnosis?

- A. Constipation
- B. Jaundice
- C. Left lower quadrant pain
- D. Polyuria

25. The nurse is assessing a patient with suspected duodenal ulcer. Which symptom would the nurse expect?

- A. Pain that is worse with food intake
- B. Pain that improves after eating or drinking milk
- C. Dull, persistent epigastric pain
- D. Jaundice and dark-colored urine

26. A client with a small bowel obstruction presents with abdominal pain, vomiting, and absent bowel sounds. What is the nurse's priority action?

- A. Encourage oral fluids to prevent dehydration
- B. Insert an NG tube as prescribed for decompression
- C. Administer a stool softener
- D. Position the client on their right side

27. The nurse is caring for a client with chronic gastritis who is at risk for vitamin B12 deficiency. What is the reason for this risk?

- A. Gastritis decreases iron absorption, leading to vitamin B12 deficiency.
- B. Chronic gastritis reduces stomach acid needed for vitamin B12 absorption.
- C. Vitamin B12 is lost due to bleeding in chronic gastritis.
- D. Increased acid production from gastritis depletes vitamin B12 levels.

28. A nurse is caring for a client with an active upper GI bleed who requires frequent monitoring. Which assessment finding should the nurse report immediately?

- A. Decreased bowel sounds in all four quadrants
- B. Increased abdominal pain with distension
- C. Urine output of 35 mL/hr
- D. Complaints of mild nausea

29. The nurse is assessing a client with suspected appendicitis. Which of the following findings is most concerning and requires immediate notification to the healthcare provider?

- A. Nausea and vomiting
- B. Elevated white blood cell (WBC) count
- C. Sudden relief of pain
- D. Fever of 99.5°F (37.5°C)

30. A client is admitted with a diagnosis of small bowel obstruction. Which of the following symptoms is most indicative of this condition?

- A. Painless rectal bleeding
- B. Gradual onset of constipation
- C. Abdominal distension with high-pitched bowel sounds
- D. Sudden, sharp abdominal pain with localized tenderness

31. A 30-year-old male presents with sudden onset of severe, diffuse abdominal pain and a rigid abdomen on examination. What is the most likely diagnosis?

- A) Acute appendicitis
- B) Perforated peptic ulcer
- C) Acute cholecystitis
- D) Bowel obstruction

32. Which of the following is a hallmark clinical manifestation of acute appendicitis?

- A) Constant epigastric pain radiating to the back
- B) Right lower quadrant pain with rebound tenderness
- C) Left upper quadrant pain with referred shoulder pain
- D) Diffuse abdominal pain with watery diarrhea

33. The nurse would increase the comfort of the patient with appendicitis by:

- a. Flexing the patient's right knee
- b. Having the patient lie prone
- c. Sitting the patient upright in a chair
- d. Turning the patient onto his or her left side

34. During removal of a fecal impaction, which of the following could occur because of vagal stimulation?

- a. Atelectasis
- b. Bradycardia
- c. Cardiac tamponade
- d. Tachycardia

35. Mr. Molla has a fecal impaction. The nurse correctly administers an oil-retention enema by doing which of the following?

- a. Mixing milk and molasses in equal parts for an enema
- b. Administering the enema while the patient is sitting on the toilet
- c. Instructing the patient to retain the enema for at least 30 minutes
- d. Administering a large volume of solution (500-1,000 mL)